

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT DEMOGRAPHICS****

Name: Patient 12

Age: 74 years old

Sex: Female

MRN: 000-TEST-12

Admission Date: [Current Date - 5 days]

Discharge Date: [Current Date]

Attending Physician: Dr. M. Richardson, MD

****PRIMARY DIAGNOSIS & COMORBIDITIES****

Primary: Acute exacerbation of Chronic Obstructive Pulmonary Disease (COPD), GOLD Stage 4 (Very Severe)

- Baseline FEV1/FVC ratio: 38% (severely reduced airflow obstruction)

- SpO2 on room air at admission: 84%; improved to 92–94% on 2L NC by discharge

Secondary Diagnoses:

- Paroxysmal atrial fibrillation (rate-controlled on current regimen)
- Obesity (BMI 34.2)
- Hypertension
- Type 2 Diabetes Mellitus

****HOSPITAL COURSE****

Patient 12 presented to the ED with acute dyspnea on exertion, increased sputum production (yellowish), and oxygen desaturation. CXR revealed hyperinflation consistent with COPD without acute infiltrate. ABG on admission: pH 7.32, pCO₂ 52 (mild hypercapnia), pO₂ 71 on RA.

Hospital course included:

- Bronchodilator nebulization (albuterol + ipratropium) q.4h initially, tapered to q.6h
- IV methylprednisolone 125 mg q.6h × 3 days, then transitioned to oral prednisone taper (see below)
- Supplemental oxygen titrated to maintain SpO₂ >90%
- EKG monitoring for AF management; HR remained 60–78 throughout stay
- Sputum culture obtained (pending at discharge; patient instructed to follow up on results)
- Incentive spirometry & pulmonary hygiene education provided

Patient demonstrated clinical improvement with reduced work of breathing, decreased sputum production, and adequate oxygenation. Discharged in stable condition on optimized regimen.

****DISCHARGE MEDICATIONS****

| Medication | Dose | Frequency | Duration/Notes |

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| ****Tiotropium**** (LAMA) | 18 mcg | Once daily via DPI | Ongoing; use Handihaler device |

| ****Formoterol/Budesonide**** (LABA/ICS combo) | 120/4.5 mcg | 2 inhalations BID | Ongoing; rinse mouth after use |

| ****Albuterol**** (SABA) | 90 mcg | 2 inhalations q.4–6h PRN | As needed for acute dyspnea |

| ****Prednisone**** (oral taper) | 40 mg q.d. × 5d, then 30 mg q.d. × 5d, then 20 mg q.d. × 5d | Taper schedule | Complete full taper; do NOT stop abruptly |

| ****Digoxin**** | 0.125 mg | Once daily (for AF rate control) | Ongoing; monitor heart rate |

Metoprolol tartrate	25 mg	BID	Ongoing
Lisinopril	10 mg	Once daily	Ongoing
Metformin	500 mg	BID with meals	Ongoing

FOLLOW-UP APPOINTMENTS

1. **Pulmonology** — Dr. S. Chen, MD at Metro Medical Center — **In 2 weeks** (06